



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : نسمة محمد سليمان محمد
المحافظة : القاهرة الحي / المدينة : المطرية
رقم البطاقة : 28509070105743 رقم التليفون : 01158197757
العنوان : الوابور
العمر (تاريخ الميلاد) : 07-09-1985 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 29-09-2026

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----) المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني الوظيفة: ----- التوقيع: -----
الاسم: د/ندى حسن الوظيفة: ----- التوقيع: -----
الاسم: د/نرمين رمضان الوظيفة: ----- التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- د. / ----- د. / -----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : ----- قسم : -----
جهة العلاج : ----- نفقات : -----
ملاحظات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: نسمه محمد سليمان محمد National ID: 28509070105743	
--	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Supportive and palliative Treatment
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



٢٠١٩/١٠
٢٨٥٠٩٠٧٠١٠٥٧٤٣
حامله على نظام تجارة
إلكترونية
مصلحة
النش
يخت الضري على القيمة مضافة
الطاقة سارية حتى ٢٠٢٩/١٠/٧



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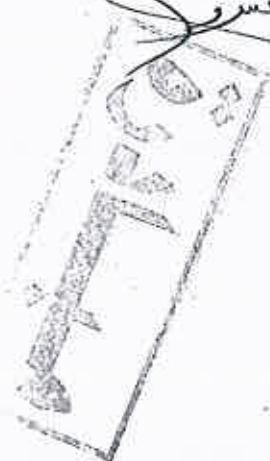


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نسبة
محمد سليمان محمد الزاهد
١٠ على قانون الضريبة على القيمة المضافة
الطرية - (٢٨٥٠٩٠٧٠١٠٥٧٤٣)
٢٨٥٠٩٠٧٠١٠٥٧٤٣

١١٠١٩٧٧٧٧

مقر مصلحة
١٩٨٥



٢٧٧٧
٢٧٧٧
٢٧٧٧

Patient Name : الأستاذ / نسمه محمد سليمان محمد : Request No. : 13005012
Patient No. : 28509070105743 : Request Date : 12/02/2026
Path No. : 422/26 : Verification Date : 23/02/2026
Sex : Female : Financial Category : نفقة الدولة - جوستاف روسي
Age : 40 Y : Department : Out Patient
Referring Doc : : Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Right cervical lymphadenopathy in patient with history of breast cancer and granuloma.

Nature of specimen: Excisional biopsy, collected on 11/02/2026.

Gross examination:

A Fibrofatty tissue piece measured 2x1.5x0.5 cm, entangling six lymph nodes, the largest is 1x0.5x0.5 cm. Totally submitted.

* 2 Blocks.

* Gross was done by; Dr. Menan Hossam.

Microscopic examination:

- Histopathological examination reveals six lymph nodes showing capsule with patent sinuses, formed of small & intermediate sized reactive lymphocytes arranged in follicles with reactive germinal centers. No granuloma. No evidence of metastatic deposits in specimen received. No evidence of malignancy in sections examined.

Diagnosis:

Cervical lymph nodes, Excisional biopsy:

Six lymph nodes with reactive follicular hyperplasia. No granuloma. No malignancy.

Signature:

Dr. Walaa Ibrahem Ghoneim.

Dr. Heba El-Shafeey.

Verified by :

Dr.Duaa Mohamed

Printing Date : 10/03/2026 13:17:56

Page 1 of 1

Printing By : marwa mohamed youss



غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

mm/dd/yyyy

MDT FORM

Name:	Nesma Mohamed Soliman Mohamed	MRN:	3737
Age:	40 Y	ID:	
Menopausal Sta		Phone:	01158197757
NRS SCORE:		Marital Status:	
BMI:		Family History:	
CO Morbidity:	TB	Reason Of MDT:	NEW CASE

Clinical Data:

BX: RT SUPRACLAVICULAR LN TRUE CUT SHOWS EXTENSIVE
ER: **PR:** **HER2:** **KI67:**

R

L

Sonomammography:

SP LT MRM POST NACT PORT HORMONAL THERAPY
H/O TB

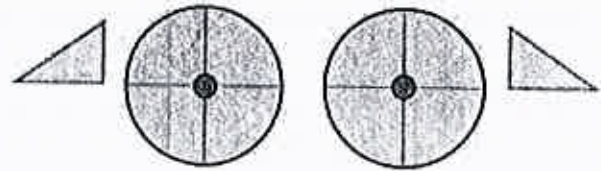
FOR EXCISIONAL BIOPSY

Mets Workup:

Recommendations:

MDT Outcome & Plan

EXCESIONAL LYMPH NODE BIOPSY FROM RT LOWER NECK SWELLING



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ خالد عبد العزيز	د/ محمد البدر				

Patient Name : الأستاذة / نسمة محمد سليمان محمد	Request No. : 13003596
Patient No. : 28509070105743	Request Date : 11/03/2024
Path No. : 1005/24	Verification Date : 24/03/2024
Sex : Female	Financial Category : نفقة الدولة - اورام دار السلام
Age : 38 Y	Department : Out Patient
Referring Doc :	Registered by : .marwa mohamed youss

Pathology Report

Clinical Data: Left breast cancer, Post NACT.

Nature of the specimen: Left MRM, collected at 10/3/2024.

Gross Examination:

- Procedure: Left MRM specimen measured 20x15x7 cm, covering skin ellipse measured 16x8 cm, nipple and areola were unremarkable grossly.
- Tumor extent
 - o Number: Ill defined grayish white TBs
 - o Size: 7x5x4 cm.
 - o Skin involvement: No
- Distance from margins: 0.5 cm from deep margin
- Lymph nodes number and size: 11 nodules up to 1.3 cm.
- Additional pathologic findings: N/A
- Immunohistochemistry technique: N/A
- Frozen section: N/A

Microscopic examination:

- Histological type: No remnants of tumor tissue
- Grade (differentiation+ nuclear pleomorphism + mitosis): N/A
- Angioinvasion: Negative
- Ductal carcinoma in situ: No
- Lymph node involvement: Free axillary lymph nodes (0/11).
- Resection Margins: Free
- Histologic response to therapy: PCR
- Additional pathologic findings: Free nipple.

Diagnosis:

Verified by :
walaa ghoniem



Patient Name	: الأستاذة / نسمة محمد سليمان محمد	Request No.	: 13003596
Patient No.	: 28509070105743	Request Date	: 11/03/2024
Path No.	: 1005/24	Verification Date	: 24/03/2024
Sex	: Female	Financial Category	: نفقة الدولة - اورام دار السلام
Age	: 38 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Left breast cancer, post NACT; Left MRM:
- No residual tumor tissue
- Free nipple
- Free deep margin
- Free axillary lymph nodes (0/11)
- ypT0 ypN0.

Dr. Rana Fathy
Dr. Walaa Ghonaim

Verified by :
walaa ghoniem

Printing Date : 30/03/2024 11:40:14

Page 2 of 2

Printing By : .marwa mohamed youss

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Patient Name: Nesma Mohamed Soliman Mohamed	Date : 21/10/2023
Exam : US Neck	Age : 38 Year(s)
Medical ID: 3868/2023	Pat. ID: 42995

Neck Ultrasonography

- The right and left lobes and isthmus of thyroid gland are of normal size, homogenous echopattern, with regular outline.
- Normal vascularity at CCDI.
- The gland moves freely with deglutition.
- The trachea is centralized.
- Both carotid sheaths are free.
- No enlarged cervical lymph nodes
- Right supraclavicular heterogenous soft tissue lesion measuring 13x8mm related to the underlying surgical scar

Conclusions

Right supraclavicular heterogenous soft tissue lesion as described for further evaluation.

Signature Dr . Nermien Ramadan

21/10/2023 2:32 PM

Presidential Program for Women's Health
Ministry of Health and Populationالبرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان

MDT form

A-Patient ID:

1- Name : نسمة محمد سليمان محمد

Sex:

3- Medical number:

Female

83167618

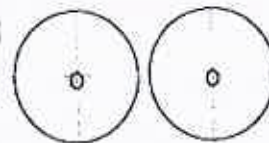
Old Med :

Old Med :

2024

2-ID number: 28509070105743

Patient Age : 38 Y



Family history

Family History

-VE

Phone

Phone

01062592976

BMI

BMI

REASON OF MDT

REASON OF MDT
POST OPERATIVE

marital status

Marital Status

NRS Score

NRS Score

GROUP

GROUP

Date of MDT

Date of MDT

16/04/2024 ☐

Co morbidity

Co Morbidity

FREE

Menopausal
statusMenopausal Status
PREMENOPAUSAL

Initial STAGE

Initial STAGE

Clinical data

Breast Imaging :

LT breast locally advanced breast + ipsilateral axillary LN

Mets Workup :

CT CAP : FREE

large adenexial lesion mostly neoplastic

Bone scan : requested

transvaginal U/S :LARGE adenxal midline & to the rt side
complex cystic lesion

BX

ER

PR

Her 2 Neu

SISH

Ki67

IDC

0/8

7/8

3

50

BX COMMENT:

ER COMMENT:

PR COMMENT:

Her 2 Neu COMMENT:

SISH COMMENT :

T

N

M

4B

1

0

Sonomammography

PATIENT HISTORY

finished 6 cycle TC + HP x4 10/2/2024

LT MRM: no residual tumor tissue

free nipple , deep margins

free ln 0/11

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

1- PORT

2- ADJUVANT TAMOXIFEN

3- ADJUVANT PHESGO

SIGNATURE

MEDICAL ONCOLOGY

MEDICAL ONCOLOGY

RADIOLOGY

RADIOLOGY

SURGERY

SURGERY

PATHOLOGY

PATHOLOGY

Psychiatric

Psychiatric

Nutrition

Nutrition

Patient Name : الأستاذة / نسمة محمد سليمان محمد ابراهيم
Patient No. : 28509070105743
Age : 3318/23
Sex : Female
Age : 38 Y
Referring Doc. :

Request No. : 13000543
Request Date : 27/09/2023
Verification Date : 21/10/2023
Financial Category : اورام دار السلام - مبادره
Department : Out Patient
IHC No. :
Registered by : Mrs.Aya Mohamed

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast mass
Nature of Specimen: Tru-cut biopsy

Technique:
Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:
PT (Positive control showed appropriate reactivity):
ER: No nuclear reaction in tumor cells (Allred score 0/8).
PR: Moderate nuclear reaction in 70% of tumor cells (Allred score 7/8).
Her2: Strong complete membrane reaction in >10% of tumor cells (score 3+).
Ki67: 50% nuclear reaction of tumor cells.

Conclusion:
Left breast mass, Tru-cut biopsy:
Invasive duct carcinoma, Grade III.

ER: Negative (score 0/8).
PR: Positive (score 7/8).
Her2: Positive (score 3+).
Ki67: Positive in 50% of tumor cells.

Scoring system: "ER, PR"
CO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0,

Verified by :



Patient Name الأستاذة / نسمة محمد سليمان محمد ابراهيم
Patient No. 28509070105743
Path No. 3318/23
Sex Female
Age 38 Y
Referring Doc.

Request No. 13000543
Request Date 27/09/2023
Verification Date 21/10/2023
Financial Category اورام دار السلام - مبادره
Department Out Patient

Registered by Mrs.Aya Mohamed

IMMUNOHISTOCHEMISTRY REPORT

Staining: 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Prof. Dr. Gamal Abdelaty
Dr. Menan Hossam

Verified by :

Printing Date : 21/10/2023 12:27:53

Page 2 of 2

Printed By : Dr.Marwa mahmoud



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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name: Nesma Mohamed Soliman Mohamed	Date : 15/10/2023
Exam : C.T CHEST WITH CONTRAST	Age : 38 Year(s)
Medical ID: 3868/2023	

- No abdominal or pelvic masses or collections.

3. **The scanned skeleton:**

- Right iliac wing and left pubic sclerotic foci mostly bone islands.
- No vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

- ***Left breast locally advanced malignant looking mass lesion with pathological ipsilateral axillary lymph nodes as described.***
- ***Bilateral scattered pulmonary nodules likely benign in nature for follow up.***
- ***No definite hepatic or osseous metastatic deposits.***
- ***Large right adnexal lesion mostly neoplastic for further assessment.***

Dr.Mohammed Mohsen, MSc.

Dr. Ola Magdy, M.D.



Patient Name: Nesma Mohamed Soliman Mohamed	Date : 20/02/2024
Exam : MAMMOGRAPHY BOTH	Age : 38 Year(s)
Medical ID: 3868/2023	Pat. ID: 42995

**DIGITAL BILATERAL SOFT TISSUE MAMMOGRAPHY AND
COMPLEMENTARY ULTRASOUND**

Indication:

Post NACT status assessment of a known left breast malignant mass

Report:

- ACR b: scattered fibro-glandular parenchyma of both breasts
- Left breast:
 - Right breast UOQ partially obscured residual dense architectural distortion and regional malignant calcifications. Ultrasound shows no definite mass
 - Residual diffuse edema pattern and skin thickening
 - Regressive course of the previously reported right axillary lymphadenopathy showing no sizable residual lymphadenopathy.
 - No supraclavicular lymphadenopathy.
- Right breast:
 - No suspicious micro calcifications or architectural distortion detected.
 - Complementary ultrasound no cystic or solid mass lesions.
 - Normal skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
 - Nonspecific axillary LN.

Opinion:

- Post NACT status of left breast showing partial response of the previously noted malignant mass with no residual lymphadenopathy (BIRADS 6)
- Normal sonomammography of right breast (BIRADS 1)

Best regards,
Dr. Amira H Radwan MD,
Breast Imaging consultant,
NCI, Cairo University
20/02/2024 12:22 PM

Patient Name: Nesma Mohamed Soliman Mohamed	Date : 21/10/2023
Exam : ultrasound TRANS VAGINAL	Age : 38 Year(s)
Medical ID: 3868/2023	Pat ID: 42995

Trans-vaginal ultrasound scanning revealed: -

○ **Uterus: -**

- Is surgically removed
- Cervical stump with average size, Nabothian cyst measuring 8mm is noted

○ **Ovaries: -**

- Large adnexal complex cystic lesion seen at the midline and to the right side is noted measuring 13x12mm. the internal septation show vascularity color score (1-2)

Conclusion:

Hysterectomy

Large adnexal midline and to the right side complex cystic lesion as described likely ovarian neoplasm (ORADS 4)for tumor markers and histopathological correlation

Signature Dr . Nermien Ramadan
21/10/2023 2:36 PM



Patient Name: Nesma Mohamed Soliman Mohamed	Date : 21/12/2024
Exam : MAMMOGRAPHY BOTH	Age : 39 Year(s)
Medical ID: 3868/2023	Pat. ID: 42995

RIGHT BREAST MAMMOGRAPHY WITH BILATERAL US

Indication:

Patient had left MRM

Technique

- right breast MLO and CC views
- Bilateral ultrasound

Report:

- The right breast is scattered fibroglandular densities of the right breast (ACR B)
- No spiculated masses or grouped microcalcifications.
- Complementary ultrasound also showed no right breast solid or cystic masses
- Normal skin thickness and nipple-areola complex, with average caliber of the retro-areolar ducts
- Nonspecific right axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free left operative bed with no evidence of recurrent mass lesions
- Free left infra and supra clavicular regions.

Opinion:

- Left MRM with free operative bed with no evidence of recurrent mass lesions
- No right breast suspicious findings (BIRADS 2)

Yours truly

Signature Dr. Yasmin Nada

21/12/2024 1:12 PM



Patient Name: Nesma Mohamed Soliman Mohamed	Date : 17/04/2024
Exam : ultrasound ABDOMEN& PELVIS	Age : 38 Year(s)
Medical ID: 3868/2023	Pat. ID: 42995

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver: -

- *Enlarged in size (17cm at Rt MCL) bright parenchymal echopattern with smooth surface.*
- *No hepatic focal lesions or intra hepatic biliary radicle dilatation.*
- *Portal vein is patent and not dilated.*

Gall Bladder: -

- *Average distention and capacity with no stones or mud*
- *The wall is not thickened.*
- *CBD is not dilated.*

Spleen: -

Average size, homogenous.

Kidneys: -

- *Both are average in size, shape and echogenicity*
- *No stones or dilated pelvicalyceal system*
- *No gross lesion*

Para-aortic and pancreatic area: *show no gross abnormalities.*

No ascites

Urinary Bladder: -

- *Average capacity with no stones or diverticular out pouching*

Uterus and adenexal : *the uterus is surgically removed
right adnexal multilocular cystic lesion measuring about
5x4.6cm*

Opinion:

*Bright hepatomegaly likely fatty
Right adnexal multilocular cystic lesion? inclusion cyst? ovarian for further
evaluation*

Signature Dr . Nermien Ramadan
PM 12:42 2024/04/17

Name: Mrs. Nesma Mohamed Soliman Mohamed

Date: 5/11/2023

ID:2223151562

Acc.:4857365

Tc-99m MDP whole body bone scan

Technique:

- Whole body images were acquired 180 minutes after IV injection of ~15 mCi of the tracer.

Findings:

- Uniform tracer uptake all over the surveyed skeleton is noted with no evidence of any active spots.
- Both kidneys are visualized.

Opinion:

- No scintigraphic evidence of metastatic osseous deposits.

Truly yours

Prof. Dr. Ahmed Kandeel. MD

Prof. of Nuclear Medicine, faculty of medicine, Cairo University

Printed by KH 06/11/23 9:08:10 AM

Patient's Name:

Nasma Mohammed Suleiman

Date:

2026-06-24

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Rousey International Egypt



Right Digital soft tissue mammography with high resolution sonography

revealed:

Clinical summary: Post left MRM & ALAND, routine follow up.

Findings:

Right breast:

- Scattered fibro-glandular densities. (ACR b)
- No speculations, masses, asymmetry, distortion or suspicious microcalcifications.

Ultrasound demonstrated no solid or cystic lesions.

- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Left mastectomy bed.

- Clear mastectomy bed with no masses or fluid collections..
- Clear left axilla. No pathological infra or supra-clavicular lymph nodes

Conclusion:-

Right breast (BIRADS 1): normal study.

Clear left mastectomy bed and axilla.

Recommendation:-

For annual follow up.

Best Regards,
DR. Amal Hegazi, M.Sc.

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined

with recommendations:

Category & recommendations

BIRADS 0: Non-informative study.

BIRADS 1: Normal study, routine screening

BIRADS 2: Benign findings, routine screening

BIRADS 3: Probably benign, for close follow up.

BIRADS 4: Probably malignant, for biopsy.

A: mild suspicion.

B: moderate suspicion.

C: high suspicion.

BIRADS 5: Malignant, for biopsy.

BIRADS 6: Pathologically proven malignant

Suspected Cancer Risk

0 %

0 %

2 %

50 %

98-100%

Patient Name : الأستاذ / نسمة محمد سليمان محمد : Request No. : 13005012
Patient No. : 28509070105743 : Request Date : 12/02/2026
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Age : 40 Y : Department : Out Patient
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* 2 Blocks.

* Gross was done by; Dr. Menan Hossam.

Microscopic examination:

- Histopathological examination reveals six lymph nodes showing capsule with patent sinuses, formed of small & intermediate sized reactive lymphocytes arranged in follicles with reactive germinal centers. No granuloma. No evidence of metastatic deposits in specimen received. No evidence of malignancy in sections examined.

Diagnosis:

Cervical lymph nodes, Excisional biopsy:

Six lymph nodes with reactive follicular hyperplasia. No granuloma. No malignancy.

Signature:

Dr. Walaa Ibrahem Ghoneim.

Dr. Heba El-Shafeey.

Verified by :

Dr.Duaa Mohamed

Printing Date : 10/03/2026 13:17:56

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٢٠١٩/١٠
٢٨٥٠٩٠٧٠١٠٥٧٤٣
حاصله على دبلوم تجارة
أنشئ مسلمة
متزوجة
بخت الزوى عبد الحميد محمد
البطاقة سارية حتى ٢٠٢٦/١٠/٠٧



ERM036725013

بطاقة تحقيق الشخصية
نسخه
محمد سليمان محمد ابراهيم
١٠ ش وابور الطحين- عزبة الواوير
المطرية - القاهرة
٢٨٥٠٩٠٧٠١٠٥٧٤٣
IF0007600

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فني ملحق شري

١٤٤٣/٥

مبارك

mm/dd/yyyy

MDT FORM

Name:	Nesma Mohamed Soliman Mohamed		
Age:	40 Y	MRN:	3737
Menopausal Sta		ID:	
NRS SCORE:		Phone:	01158197757
BMI:		Marital Status:	
CO Morbidity:	TB	Family History:	
		Reason Of MDT:	NEW CASE

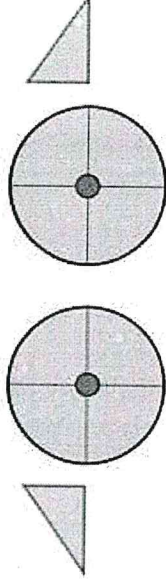
Clinical Data:

BX: RT SUPRACLAVICULAR LN TRUE CUT SHOWS EXTENSIVE
ER: **PR:** **HER2:** **KI67:**

Sonomammography:
 SP LT MRM POST NACT PORT HORMONAL THERAPY
 H/O TB
 FOR EXCISIONAL BIOPSY
Mets Workup:
Recommendations:
MDT Outcome & Plan
 EXCESIONAL LYMPH NODE BIOPSY FROM RT LOWER NECK SWELLING

R

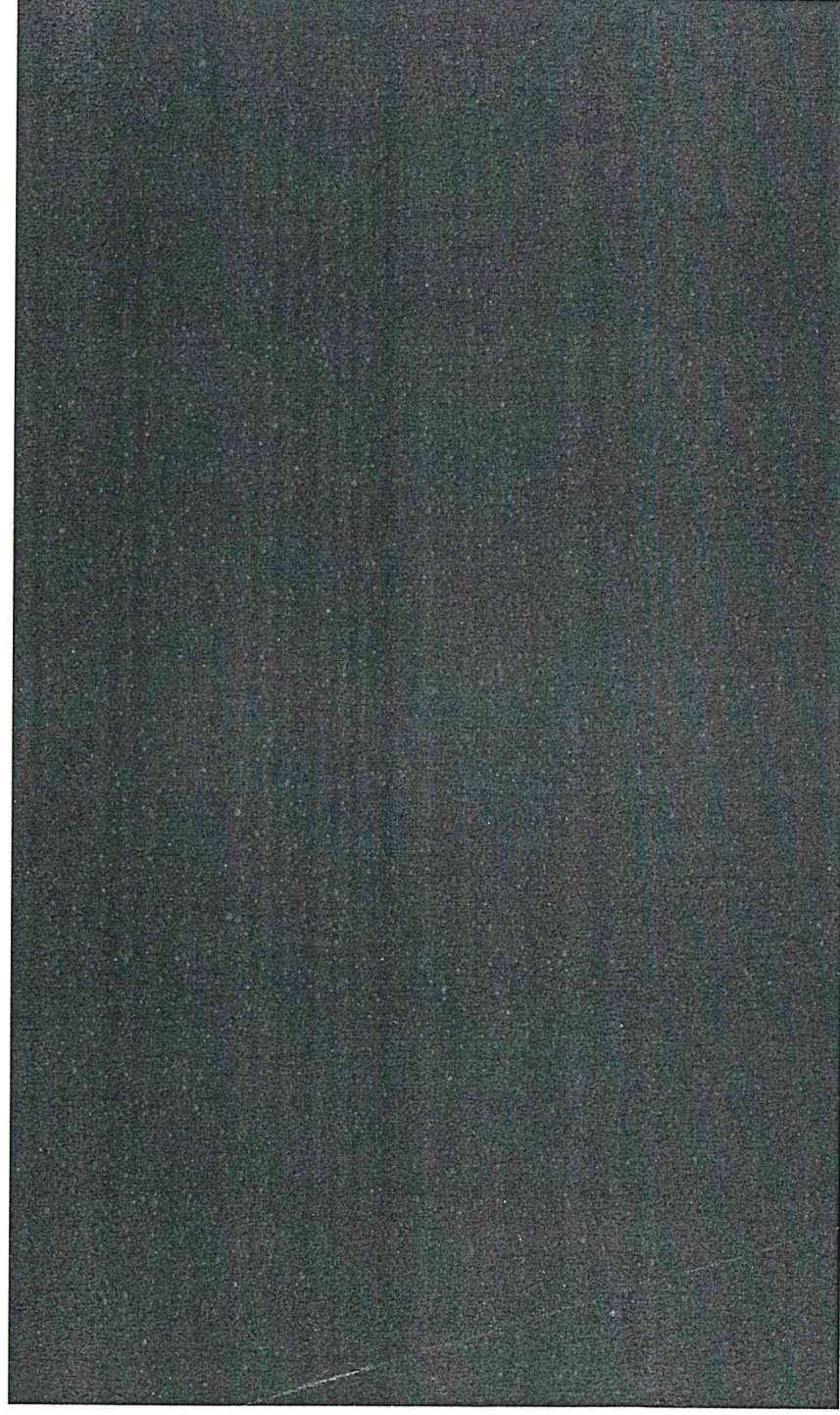
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T N M

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ خالد عبد العزيز	د/ محمد النوراني				





Patient Name : الأستاذة / نسمة محمد سليمان محمد : Request No. : 13003596
Patient No. : 28509070105743 : Request Date : 11/03/2024
Path No. : 1005/24 : Verification Date : 24/03/2024
Sex : Female : Financial Category : نفقة الدولة - اورام دار السلام
Age : 38 Y : Department : Out Patient
Referreing Doc : : Registered by : .marwa mohamed youss

Pathology Report

Clinical Data: Left breast cancer, Post NACT.

Nature of the specimen: Left MRM, collected at 10/3/2024.

Gross Examination:

- Procedure: Left MRM specimen measured 20x15x7 cm, covering skin ellipse measured 16x8 cm, nipple and areola were unremarkable grossly.
- Tumor extent
 - o Number: ill dfined grayish white TBs
 - o Size: 7x5x4 cm.
 - o Skin involvement: No
 - o Distance from margins: 0.5 cm from deep margin
 - o Lymph nodes number and size: 11 nodules up to 1.3 cm.
- Additional pathologic findings: N/A
- Immunohistochemistry technique: N/A
- Frozen section: N/A

Microscopic examination:

- Histological type: No remnants of tumor tissue
- Grade (differentiation+ nuclear pleomorphism + mitosis): N/A
- Angioinvasion: Negative
- Ductal carcinoma in situ: No
- Lymph node involvement: Free axillary lymph nodes (0/41).
- Resection Margins: Free
- Histologic response to therapy: PCR
- Additional pathologic findings: Free nipple.

Diagnosis:

Verified by :
walaa ghoniem

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Patient Name: Nesma Mohamed Soliman Mohamed	Date : 21/10/2023
Exam : US Neck	Age : 38 Year(s)
Medical ID: 3868/2023	Pat. ID: 42995

Neck Ultrasonography

- The right and left lobes and isthmus of thyroid gland are of normal size, homogenous echopattern, with regular outline.
- Normal vascularity at CCDL.
- The gland moves freely with deglutition.
- The trachea is centralized.
- Both carotid sheaths are free.
- No enlarged cervical lymph nodes
- Right supraclavicular heterogenous soft tissue lesion measuring 13x8mm related to the underlying surgical scar

Conclusions

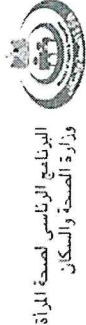
Right supraclavicular heterogenous soft tissue lesion as described for further evaluation.

Signature Dr . Nermien Ramadan

21/10/2023 2:32 PM



Presidential Program for Women's Health
Ministry of Health and Population



MDT form

A-Patient ID:

1- Name : نسمة محمد سليمان محمد

Sex:

Female

3- Medical number:

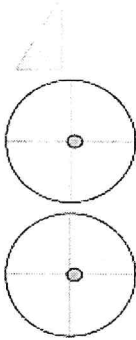
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Old Med :

Old Med : 2024

2-ID number: 28509070105743

Patient Age : 38 Y



Family history	Family History -VE	Phone	01062592976	BMI	BMI	REASON OF MDT POST OPERATIVE
marital status	Marital Status	NRS Score	NRS Score	GROUP	GROUP	Date of MDT
Co morbidity	Co Morbidity FREE	Menopausal status	Menopausal Status PREMENOPAUSE	Initial STAGE	Initial STAGE	16 / 04 / 2024

Clinical data	
Breast Imaging :	Mets Workup :
LT breast locally advanced breast + ipsilateral axillary LN	CT CAP : FREE
	large adenexial lesion mostly neoplastic
	Bone scan : requested
	transvaginal U/S :LARGE adenxal midline & to the rt side
	complex cystic lesion

BX IDC	ER 0/8	PR 7/8	Her 2 Neu 3	SISH	Ki67 50
BX COMMENT:	ER COMMENT:	PR COMMENT:	Her 2 Neu COMMENT:	SISH COMMENT :	

T 4B	N 1	M 0
------	-----	-----

Sonomammography

PATIENT HISTORY

finished 6 cycle TC + HP x4 10/2/2024

LT MRM: no residual tumor tissue

free nipple , deep margins

free ln 0/11

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

1- PORT

2- ADJUVANT TAMOXIFEN

3- ADJUVANT PHESGO

SIGNATURE		
MEDICAL ONCOLOGY	MEDICAL ONCOLOGY	RADIOLOGY
SURGERY	SURGERY	PATHOLOGY
Psychiatric	Psychiatric	Nutrition



Patient Name	: الأستاذة / نسمة محمد سليمان محمد ابراهيم	Request No.	: 13000543
Ident No.	: 28509070105743	Request Date	: 27/09/2023
Age	: 3318/23	Verification Date	: 21/10/2023
Sex	: Female	Financial Category	: أورام دار السلام - مبادره
Age	: 38 Y	Department	: Out Patient
Referring Doc.	:	IHC No.	:
		Registered by	: Mrs.Aya Mohamed

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast mass
Nature of Specimen: Tru-cut biopsy

Technique:

Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and DAB as counter stain. The sections were treated against:
Her2 clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:

T (Positive control showed appropriate reactivity):

- Her2: No nuclear reaction in tumor cells (Allred score 0/8).
- PR: Moderate nuclear reaction in 70% of tumor cells (Allred score 7/8).
- Her2: Strong complete membrane reaction in >10% of tumor cells (score 3+).
- Her2: 50% nuclear reaction of tumor cells.

Conclusion:

Left breast mass, Tru-cut biopsy:
Invasive duct carcinoma, Grade III.

- Her2: Negative (score 0/8).
- PR: Positive (score 7/8).
- Her2: Positive (score 3+).
- Her2: Positive in 50% of tumor cells.

Scoring system: "ER, PR"

CO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0,

Verified by :

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Patient Name: Nesma Mohamed Soliman Mohamed	Date : 15/10/2023
Exam : C.T CHEST WITH CONTRAST	Age : 38 Year(s)
Medical ID: 3868/2023	

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN & PELVIS REVEALED: -**

1. Chest:

- left breast UOQ irregular speculated enhancing mass lesion seen reaching the retracted nipple and thickened skin, it is measuring 7 X 3 cm.
- left axillary level I and II pathologically enlarged lymph nodes the largest measures 2.5 X 2 cm.
- Few bilateral scattered tiny pulmonary nodules some of them show internal calcifications, the largest is noted at apical segment of right upper lung lobe measuring 5 mm.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. Abdomen and pelvis:

- The liver is average in size with homogenous CT texture. No definite focal lesions or contour irregularities.
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Right adnexal cystic lesion with internal enhancing septations is seen measuring 8 X 13 cm.
- Normal appearance of the other pelvic organs.



Patient Name: Nesma Mohamed Soliman Mohamed	Date : 20/02/2024
Exam : MAMMOGRAPHY BOTH	Age : 38 Year(s)
Medical ID: 3868/2023	Pat. ID: 42995

DIGITAL BILATERAL SOFT TISSUE MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

Indication:

Post NACT status assessment of a known left breast malignant mass

Report:

- ACR b: scattered fibro-glandular parenchyma of both breasts
- Left breast:
 - Right breast UOQ partially obscured residual dense architectural distortion and regional malignant calcifications. Ultrasound shows no definite mass
 - Residual diffuse edema pattern and skin thickening
 - Regressive course of the previously reported right axillary lymphadenopathy showing no sizable residual lymphadenopathy.
 - No supraclavicular lymphadenopathy.
- Right breast:
 - No suspicious micro calcifications or architectural distortion detected.
 - Complementary ultrasound no cystic or solid mass lesions.
 - Normal skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
 - Nonspecific axillary LN.

Opinion:

- Post NACT status of left breast showing partial response of the previously noted malignant mass with no residual lymphadenopathy (BIRADS 6)
- Normal sonomammography of right breast (BIRADS 1)

Best regards,

Dr. Amira H Radwan MD,
Breast Imaging consultant,
NCI, Cairo University

20/02/2024 12:22 PM

Patient Name: Nesma Mohamed Soliman Mohamed	Date : 21/10/2023
Exam : ultrasound TRANS VAGINAL	Age : 38 Year(s)
Medical ID: 3868/2023	Pat. ID: 42995

Trans-vaginal ultrasound scanning revealed: -

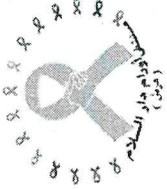
- Uterus: -
 - Is surgically removed
 - Cervical stum with average size, Nabothian cyst measuring 8mm is noted
- Ovaries: -
 - Large adnexal complex cystic lesion seen at the midline and to the right side is noted measuring 13x12mm. the internal septation show vascularity color score (1-3)

Conclusion:

Hysterectomy

Large adnexal midline and to the right side complex cystic lesion as described likely ovarian neoplasm (ORADS 4)for tumor markers and histopathological correlation

Signature Dr . Nermien Ramadan
21/10/2023 2:36 PM



Patient Name: Nesma Mohamed Soliman Mohamed	Date : 21/12/2024
Exam : MAMMOGRAPHY BOTH	Age : 39 Year(s)
Medical ID: 3868/2023	Pat. ID: 42995

RIGHT BREAST MAMMOGRAPHY WITH BILATERAL US

Indication:

Patient had left MRM

Technique

- right breast MLO and CC views
- Bilateral ultrasound

Report:

- The right breast is scattered fibroglandular densities of the right breast (ACR B)
- No spiculated masses or grouped microcalcifications.
- Complementary ultrasound also showed no right breast solid or cystic masses
- Normal skin thickness and nipple-areola complex, with average caliber of the retro-areolar ducts
- Nonspecific right axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free left operative bed with no evidence of recurrent mass lesions
- Free left infra and supra clavicular regions.

Opinion:

- Left MRM with free operative bed with no evidence of recurrent mass lesions
- No right breast suspicious findings (BIRADS 2)

Yours truly

Signature Dr. Yasmin Nada

21/12/2024 1:12 PM



Patient Name: Nesma Mohamed Soliman Mohamed	Date : 17/04/2024
Exam : ultrasound ABDOMEN& PELVIS	Age : 38 Year(s)
Medical ID: 3868/2023	Pat. ID: 42995

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver: -

- Enlarged in size (17cm at Rt MCL) bright parenchymal echopattern with smooth surface.
- No hepatic focal lesions or intra hepatic biliary radicle dilatation.
- Portal vein is patent and not dilated.

Gall Bladder: -

- Average distention and capacity with no stones or mud
- The wall is not thickened.
- CBD is not dilated.

Spleen: -

Average size, homogenous.

Kidneys: -

- Both are average in size, shape and echogenicity
- No stones or dilated pelvicalyceal system
- No gross lesion

Para-aortic and pancreatic area: show no gross abnormalities.

No ascites

Urinary Bladder: -

- Average capacity with no stones or diverticular out pouching

Uterus and adenexal; the uterus is surgically removed right adnexal multilocular cystic lesion measuring about 5x4.6cm

Opinion:

Bright hepatomegaly likely fatty

Right adnexal multilocular cystic lesion? inclusion cyst? ovarian for further evaluation

Signature Dr . Nermien Ramadan
PM 12:42 2024/04/17

Name: Mrs. Nesma Mohamed Soliman Mohamed

Date: 5/11/2023

ID: 2223151562

Acc.: 4857365

Tc-99m MDP whole body bone scan

Technique:

- Whole body images were acquired 180 minutes after IV injection of ~15 mCi of the tracer.

Findings:

- Uniform tracer uptake all over the surveyed skeleton is noted with no evidence of any active spots.
- Both kidneys are visualized.

Opinion:

- No scintigraphic evidence of metastatic osseous deposits.*

Truly yours

Prof. Dr. Ahmed Kandeel. MD

Prof. of Nuclear Medicine, faculty of medicine, Cairo University

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Patient Name	الأستاذة / نسمة محمد سليمان محمد ابراهيم	Request No.	13000543
Patient No.	28509070105743	Request Date	27/09/2023
Path No.	3318/23	Verification Date	21/10/2023
Sex	Female	Financial Category	اورام دار السلام - مبادره
Age	38 Y	Department	Out Patient
Referring Doc.		Registered by	Mrs.Aya Mohamed

IMMUNOHISTOCHEMISTRY REPORT

Staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature

Prof. Dr. Gamal Abdelaty
Dr. Menan Hossam

Verified by :

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Page 2 of 2

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MDT FORM / / 20 G(A)



SIGNATURE					
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric



Patient Name : الأستاذ / نسمة محمد سليمان محمد : Request No. : 13003142
Patient No. : 28509070105743 : Request Date : 17/02/2025
Path No. : 686/25 : Verification Date : 09/03/2025
Sex : Female : Financial Category : نفقة الدولة - اورام دار السلام
Age : 39 Y : Department : Out Patient
Referreing Doc : : Registered by : Mrs.Mayada Hegazy

Pathology Report

sections examined.

- Para tubal cyst
- Ovarian tissue with hemorrhagic corpus leuteum cyst
- Congested omentum
- histopathological examination of sections prepared from the appendicectomy specimen received shows partially ulcerated mucosa, congested oedematous submucosa and hyperplastic submucosal lymphoid follicles. There is no evidence of specific inflammation or neoplasia in sections examined.
- Hemorrhagic inetestinal mucosa with angiodysplasia
- No malignancy

Diagnosis

Right ovarian mass, excision, &ometectomy &appendectomy :-Ovarian serous

- cystadenoma
- Paratubal cyst
- Ovarian tissue with hemorrhagic corpus leutum
- C ngested omentum
- Follicular appendicitis
- Hemorrhagic intestinal mucosa with angio dysplasia
- No malignancy

Signature

Dr. Rana Fathy

Dr.Heba Hassan

Verified by :

Dr.Heba Hasan

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Patient Name	الاستاذ / نسمه محمد سليمان محمد	Request No.	: 13003142
Patient No.	: 28509070105743	Request Date	: 17/02/2025
Path No.	: 686/25	Verification Date	: 09/03/2025
Sex	: Female	Financial Category	: نفقة الدولة - اورام دار السلام
Age	: 39 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical data

Ovarian mass

Nature of specimen

Ovarian mass excision, ometectomy & appendectomy , small bowel resection

Gross specimen

Four specimens were received :

- 1) Right ovarian mass : an intact bilocular cyst measured 9.5x5x4.5 cm, outer surface was focally rough , on opening , it contains yellowish serous fluid , the lining was smooth showing cystification , wall thickness 0.3 cm, the outer surface showed disrupted fimbriated tubal segments measured 3.5 cm , also received in the same container multiple irregular tissue pieces measured collectively 5x2.5 cm , with grey brown & rim of glodedn yellow cut section
- 2) Omental tissue piece , two omental tissue pieces measured 8x4 cm, and 4.5 x2 cm with no gross nodules or induration
- 3) Appendectomy specimen measured 4.5 long , ouetr surface was smooth & glistening , lumen was patent , maximum diameter 0.4 cm, average wall thickness 0.3 c,
- 4) Small intestinal segement measured 12 cm long with one end diameter 2 cm , the other end 1.5 cm , ouer surface was rough and congested , showed mutple perforations , ranging in size from 1.5x1 cm , upto 2.5x2 cm, the smallest perforationn located 0.4 cm from narrower end, on opening , the mucosa showed thinned out area measured 2x2 cm, located 3 cm from wider end with hemorrhagic mucosal cystification

Microscopic examination
-histopathological examination of sections prepared from the ovarian cystectomy received reveals a serous cyst lined by low cuboidal cells. The wall shows small papillae with broad fibrous cores covered by same type of epithelium. There is no evidence of malignancy in

Verified by :

Dr.Heba Hasan

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Printing Date : 15/03/2025 11:36:37 Page 1 of 3



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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



Patient Name: Nesma Mohamed Soliman Mohamed	Date : 21/12/2024
Exam : C.T CHEST WITH CONTRAST	Age : 39 Year(s)
Medical ID: 3868/2023	Pat. ID: 42995

- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- **Progressive course** regading the size and extensions of the multiloculated right adnexal cystic lesion with internal enhancing septations seen currently measuring 9x9.7x14 in maximum AP, TS, and CC dimensions respectively compared to 5x5.6x9.5cm priorly.
- Normal appearance of the other pelvic organs.

The scanned skeleton:

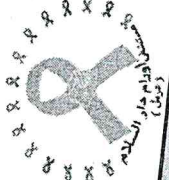
- Still noted right iliac wing and left pubic sclerotic foci mostly bone islands.
- L2 small sclerotic foci **for bone scan correlation.**
- No vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

- **Progressive course regading right adnexal lesion.**
- **Stationary course of the rest of the study.**

Dr. Basma Mohamed

Dr. Walid Mohamed, M.D.



Patient Name: Nesma Mohamed Soliman Mohamed	Date : 21/12/2024
Exam : C.T CHEST WITH CONTRAST	Age : 39 Year(s)
Medical ID: 3868/2023	Pat. ID: 42995

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

*****Breast Cancer patient underwent left mastectomy. ** As Compared to previous study dated (13/6/2024)***

Findings:

I Chest: -

- Clear operative bed showing no masses or cysts.
- Still noted:
 - Bilateral apical fine reticulations, and bilateral scattered atelectatic bands
 - Few bilateral scattered tiny pulmonary nodules some of them show internal calcifications, the largest is noted at apical segment of right upper lung lobe measuring 5 mm.
- Currently seen left upper lobe subpleural atelectatic bands and fine reticulations, Likely post radiation sequel.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- The liver is average in size with homogenous CT texture. No definite focal lesions or contour irregularities.
- Patent homogenously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.

Patient Name : الأستاذ / نعمة محمد سليمان محمد
Patient No. : 28509070105743
Path No. : 3895/25
Sex : Female
Age : 40 Y
Referreing Doc :
Request No. : 13004035
Request Date : 18/12/2025
Verification Date : 22/12/2025
Financial Category : نفقة الدولة - جوستاف روسي
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical data

History of breast cancer, underwent MRM, with history of T.B.

Nature of specimen

Tru cut biopsy from right suraclavicular lymph node, collected on 4:10 p.m , cold ischemia time :6 seconds.

Gross specimen

Tiny grey white tissue core measured collectively 0.1x0.1 cm, totally submitted.

Microscopic examination &Diagnosis:

Right supraciavicular lymph node:

Extensively necrotized tissue with calcification.

Differential diagnosis includes necrotizing granulomas or fungal infection.

-Excision biopsy is mandatory for proper diagnosis.

Signature

Prof. Dr. Maha Guimei
Dr.Walaa Ibrahim Ghoneim

Verified by :

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Page 1 of 1

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name: Nesma Mohamed Soliman Mohamed	Date : 16/09/2025
Exam : MAMMOGRAPHY BOTH	Age : 40 Year(s)
Medical ID: 3868/2023	Pat. ID: 42995

Right Digital soft tissue mammography with high resolution sonography revealed:

Clinical summary: Post left MRM & ALAND, routine follow up.

Findings:

Right breast.

- Scattered fibro-glandular densities. (ACR b)
- No speculations, masses, asymmetry, distortion or suspicious microcalcifications.
- Ultrasound demonstrated no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Left mastectomy bed.

- Clear mastectomy bed with no masses or fluid collections..
- Clear left axilla. No pathological infra or supra-clavicular lymph nodes

Conclusion:-

Right breast (BIRADS 1): normal study.

Clear left mastectomy bed and axilla.

Recommendation:-

For annual follow up.

Best Regards,
DR. Amal Hegazi, M.Sc.

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with recommendations:

Category & recommendations

	Suspected Cancer Risk
BIRADS 0: Non-informative study.	
BIRADS 1: Normal study, routine screening	0 %
BIRADS 2: Benign findings, routine screening	0 %
BIRADS 3: Probably benign, for close follow up.	2 %
BIRADS 4: Probably malignant, for biopsy.	50 %
A: mild suspicion.	
B: moderate suspicion.	
C: high suspicion.	
BIRADS 5: Malignant, for biopsy.	98-100%
BIRADS 6: Pathologically proven malignant	

Signature Dr. Amal Hegazy
PM 2:54 2025/09/16

Patient Name: Nesma Mohamed Soliman Mohamed	Date : 21/09/2025
Exam : ultrasound TRANS VAGINAL	Age : 40 Year(s)
Medical ID: 3868/2023	Pat. ID: 42995

Trans-vaginal ultrasound scanning revealed: -

- **Uterus: -**
 - Surgically removed
 - Cervical stump measuring about 26x20mm showing no focal lesions
- **Right adnexal multilocular cyst measuring about 30x35mm with no vascularity seen inside for follow up**
- **No free fluid at the pelvis**

Signature Dr . Nermien Ramadan
21/09/2025 12:12 PM

Patient Name: Nesma Mohamed Soliman Mohamed		Date : 21/09/2025
Exam : US Neck		Age : 40 Year(s)
Medical ID: 3868/2023		Pat. ID: 42995

Neck Ultrasonography

- *The right and left lobes and isthmus of thyroid gland are of normal size, homogenous echo pattern, with regular outline.*
- *Normal vascularity at CCDI.*
- *The gland moves freely with deglutition.*
- *The trachea is centralized.*
- *Both carotid sheaths are free.*
- *No pathologically enlarged cervical LNs*

Conclusions

Unremarkable study

Signature Dr . Nermien Ramadan

21/09/2025 12:10 PM

٢٠١٩/١٠ ٢٨٥٠٩٠٧٠١٠٥٧٤٣
 حاصله على دبلوم تجارة
 أنثى مسلمة
 بخيت الضوى عبدالحميد محمد
 البطاقة سارية حتى ٢٠٢٦/١٠/٠٧



ERM036725013

بطاقة تحقيق الشخصية
 نسبه
 محمد سنيمان محمد ابراهيم
 ١٠ ش وابور الطحين- عزبة الوابور
 المطرية - القاهرة
 ٢٨٥٠٩٠٧٠١٠٥٧٤٣
 IF0007600

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 ٢٨٦٨

فوتو ملحق شى

٤/٥

مباركة



Patient Name	: الأستاذة / نسمة محمد سليمان محمد	Request No.	: 13003596
Patient No.	: 28509070105743	Request Date	: 11/03/2024
Path No.	: 1005/24	Verification Date	: 24/03/2024
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 38 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Left breast cancer, Post NACT.

Nature of the specimen: Left MRM, collected at 10/3/2024.

Gross Examination:

- Procedure: Left MRM specimen measured 20x15x7 cm, covering skin ellipse measured 16x8 cm, nipple and areola were unremarkable grossly.
- Tumor extent
 - o Number: Ill dfined grayish white TBs
 - o Size: 7x5x4 cm.
 - o Skin involvement: No
- Distance from margins: 0.5 cm from deep margin
- Lymph nodes number and size: 11 nodules up to 1.3 cm.
- Additional pathologic findings: N/A
- Immunohistochemistry technique: N/A
- Frozen section: N/A

Microscopic examination:

- Histological type: No remnants of tumor tissue
- Grade (differentiation+ nuclear pleomorphism + mitosis): N/A
- Angioinvasion: Negative
- Ductal carcinoma in situ: No
- Lymph node involvement: Free axillary lymph nodes (0/11).
- Resection Margins: Free
- Histologic response to therapy: PCR
- Additional pathologic findings: Free nipple.

Diagnosis:

Verified by :
walaa ghoniem



Patient Name : الأستاذة / نسمة محمد سليمان محمد ابراهيم
 Patient No. : 28509070105743
 Age : 3318/23
 Sex : Female
 Age Y : 38 Y
 Referring Doc. :

Request No. : 13000543
 Request Date : 27/09/2023
 Verification Date : 21/10/2023
 Financial Category : اورام دار السلام - مبادره
 Department : Out Patient
 IHC No. :
 Registered by : Mrs.Aya Mohamed

IMMUNOHISTOCHEMISTRY REPORT

Initial Data: Left breast mass
 Nature of Specimen: Tru-cut biopsy

Technique:

Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
 - ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:

* IPT (Positive control showed appropriate reactivity):
 - ER: No nuclear reaction in tumor cells (Allred score 0/8).
 - PR: Moderate nuclear reaction in 70% of tumor cells (Allred score 7/8).
 - Her2: Strong complete membrane reaction in >10% of tumor cells (score 3+).
 - Ki67: 50% nuclear reaction of tumor cells.

Conclusion:

Left breast mass, Tru-cut biopsy:
 Invasive duct carcinoma, Grade III.
 IPT:

- ER: Negative (score 0/8).
 - PR: Positive (score 7/8).
 - Her2: Positive (score 3+).
 - Ki67: Positive in 50% of tumor cells.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0,

Verified by :



Patient Name: Nesma Mohamed Soliman Mohamed	Date : 15/10/2023
Exam : C.T CHEST WITH CONTRAST	Age : 38 Year(s)
Medical ID: 3868/2023	

- No abdominal or pelvic masses or collections.

3. The scanned skeleton:

- Right iliac wing and left pubic sclerotic foci mostly bone islands.
- No vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

- ***Left breast locally advanced malignant looking mass lesion with pathological ipsilateral axillary lymph nodes as described.***
- ***Bilateral scattered pulmonary nodules likely benign in nature for follow up.***
- ***No definite hepatic or osseous metastatic deposits.***
- ***Large right adnexal lesion mostly neoplastic for further assessment.***

Dr.Mohammed Mohsen, MSc.

Dr. Ola Magdy, M.D.



Patient Name: Nesma Mohamed Soliman Mohamed	Date : 15/10/2023
Exam : C.T CHEST WITH CONTRAST	Age : 38 Year(s)
Medical ID: 3868/2023	

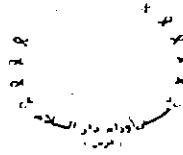
**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN & PELVIS REVEALED: -**

1. Chest:

- left breast UOQ irregular speculated enhancing mass lesion seen reaching the retracted nipple and thickened skin, it is measuring 7 X 3 cm.
- left axillary level I and II pathologically enlarged lymph nodes the largest measures 2.5 X 2 cm.
- Few bilateral scattered tiny pulmonary nodules some of them show internal calcifications, the largest is noted at apical segment of right upper lung lobe measuring 5 mm.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. Abdomen and pelvis:

- The liver is average in size with homogenous CT texture. No definite focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Right adnexal cystic lesion with internal enhancing septations is seen measuring 8 X 13 cm.
- Normal appearance of the other pelvic organs.



Patient Name: Nesma Mohamed Soliman Mohamed	Date : 20/02/2024
Exam : MAMMOGRAPHY BOTH	Age : 38 Year(s)
Medical ID: 3868/2023	Pat. ID: 42995

DIGITAL BILATERAL SOFT TISSUE MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

Indication:

Post NACT status assessment of a known left breast malignant mass

Report:

- ACR b: scattered fibro-glandular parenchyma of both breasts
- Left breast:
 - Right breast UOQ partially obscured residual dense architectural distortion and regional malignant calcifications. Ultrasound shows no definite mass
 - Residual diffuse edema pattern and skin thickening
 - Regressive course of the previously reported right axillary lymphadenopathy showing no sizable residual lymphadenopathy.
 - No supraclavicular lymphadenopathy.
- Right breast:
 - No suspicious micro calcifications or architectural distortion detected.
 - Complementary ultrasound no cystic or solid mass lesions.
 - Normal skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
 - Nonspecific axillary LN.

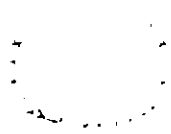
Opinion:

- Post NACT status of left breast showing partial response of the previously noted malignant mass with no residual lymphadenopathy (BIRADS 6)
- Normal sonomammography of right breast (BIRADS 1)

Best regards,

Dr. Amira H Radwan MD,
Breast Imaging consultant,
NCI, Cairo University

20/02/2024 12:22 PM



Patient Name: Nesma Mohamed Soliman Mohamed	Date : 21/10/2023
Exam : ultrasound TRANS VAGINAL	Age : 38 Year(s)
Medical ID: 3868/2023	Pat. ID: 42995

Trans-vaginal ultrasound scanning revealed: -

↳ **Uterus: -**

- Is surgically removed
- Cervical stump with average size, Nabothian cyst measuring 8mm is noted

↳ **Ovaries: -**

- ↳ Large adnexal complex cystic lesion seen at the midline and to the right side is noted measuring 13x12mm. the internal septation show vascularity color score (1-2)

Conclusion:

Hysterectomy

Large adnexal midline and to the right side complex cystic lesion as described likely ovarian neoplasm (ORADS 4)for tumor markers and histopathological correlation

Signature Dr . Nermien Ramadan
21/10/2023 2:36 PM



Patient Name: Nesma Mohamed Soliman Mohamed	Date : 21/12/2024
Exam : MAMMOGRAPHY BOTH	Age : 39 Year(s)
Medical ID: 3868/2023	Pat. ID: 42995

RIGHT BREAST MAMMOGRAPHY WITH BILATERAL US

Indication:

Patient had left MRM

Technique

- right breast MLO and CC views
- Bilateral ultrasound

Report:

- The right breast is scattered fibroglandular densities of the right breast (ACR B)
- No spiculated masses or grouped microcalcifications.
- Complementary ultrasound also showed no right breast solid or cystic masses
- Normal skin thickness and nipple-areola complex, with average caliber of the retro-areolar ducts
- Nonspecific right axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free left operative bed with no evidence of recurrent mass lesions
- Free left infra and supra clavicular regions.

Opinion:

- Left MRM with free operative bed with no evidence of recurrent mass lesions
- No right breast suspicious findings (BIRADS 2)

Yours truly

Signature Dr. Yasmin Nada

21/12/2024 1:12 PM



Patient Name: Nesma Mohamed Soliman Mohamed	Date : 17/04/2024
Exam : ultrasound ABDOMEN& PELVIS	Age : 38 Year(s)
Medical ID: 3868/2023	Pat. ID: 42995

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver: -

- Enlarged in size (17cm at Rt MCL) bright parenchymal echopattern with smooth surface.
- No hepatic focal lesions or intra hepatic biliary radicle dilatation.
- Portal vein is patent and not dilated.

Gall Bladder: -

- Average distention and capacity with no stones or mud
- The wall is not thickened.
- CBD is not dilated.

Spleen: -

Average size, homogenous.

Kidneys: -

- Both are average in size, shape and echogenicity
- No stones or dilated pelvicalyceal system
- No gross lesion

Para-aortic and pancreatic area: show no gross abnormalities.

No ascites

Urinary Bladder: -

- Average capacity with no stones or diverticular out pouching

Uterus and adenexal: the uterus is surgically removed
right adnexal multilocular cystic lesion measuring about
5x4.6cm

Opinion:

Bright hepatomegaly likely fatty

Right adnexal multilocular cystic lesion? inclusion cyst? ovarian for further
evaluation

Signature Dr . Nermien Ramadan
PM 12:42 2024/04/17

Name: Mrs. Nesma Mohamed Soliman Mohamed

Date: 5/11/2023

ID:2223151562

Acc.:4857365

Tc-99m MDP whole body bone scan

Technique:

- Whole body images were acquired 180 minutes after IV injection of ~15 mCi of the tracer.

Findings:

- Uniform tracer uptake all over the surveyed skeleton is noted with no evidence of any active spots.
- Both kidneys are visualized.

Opinion:

- No scintigraphic evidence of metastatic osseous deposits.*

Truly yours

Prof. Dr. Ahmed Kandeel. MD

Prof. of Nuclear Medicine, faculty of medicine, Cairo University

Printed by KH 06/11/23 9:08:10 AM



Patient Name	الأستاذة / نسمة محمد سليمان محمد ابراهيم	Request No.	13000543
Patient No.	28509070105743	Request Date	27/09/2023
Path No.	3318/23	Verification Date	21/10/2023
Sex	Female	Financial Category	اورام دار السلام - مبادره
Age	38 Y	Department	Out Patient
Referring Doc.			
		Registered by	Mrs.Aya Mohamed

IMMUNOHISTOCHEMISTRY REPORT

no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature

Prof. Dr. Gamal Abdelaty

Dr. Menan Hossam

Verified by :

Printing Date : 21/10/2023 12:27:53

Page 2 of 2

Printed By : Dr.Marwa mahmoud



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



Echo report

Name: نور
Date: 17/9 2024

Age: 28

EF	69%	54-74%	PWT	0.7	0.6-1.1cm
LVEDD	4.2	3.5-5.7cm	AO	2.1	2.0-3.7cm
LVESD		2.2-4.0cm	LA	3.1	1.9-4.0cm
FS		25-45%	RV		0.7-2.3cm (supine)
SWT	0.9	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV dimensions: Normal

LV wall thickness: Normal

LV diastolic function: Normal

SWMA. normal

LA size normal

Aortic root dimension normal

Mitral Valve normal

Aortic valve normal

Tricuspid valve normal

Pulmonary valve normal

Right side size and systolic function normal

Intracardiac masses and thrombi normal

Pericardium normal

IVC diameter and collapsibility. normal

Conclusion:

Dr:

د/مينا عبد



Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



MDT form

A-Patient ID:

1- Name : نسمة محمد سليمان محمد

Sex:

3- Medical number:

Female

83167618

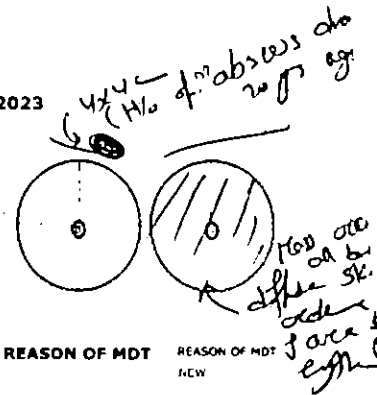
Old Med :

Old Med :

3868/2023

2-ID number: 28509070105743

Patient Age : 38 Y



Family History

Family History
-VE

Phone

Phone

01158197717

BMI

BMI

REASON OF MDT

REASON OF MDT
NEW

Marital status

Marital Status

NRS Score

NRS Score

GROUP

GROUP

Date of MDT

Date Of MDT

2023-10-25

Co morbidity

Co Morbidity

Menopausal
status

Menopausal Status

Clinical data

Breast Imaging :

LT breast locally advanced breast + ipsilateral axillary LN

Mets Workup :

CT CAP : FREE

large acenexial lesion mostly neoplastic

Bone scan : requested / transvaginal U/S requested neck U/S

BX:

IDC

Bx COMMENT

GII

ER:

0/8

ER COMMENT

PR:

7/8

PR COMMENT

Her 2 Neu :

3

Her 2 Neu COMMENT

SISH :

SISH COMMENT

Ki67 :

50%

T

4b

N

1

M

0

Sonomammography

PATIENT HISTORY

1.MRI abd & pelvis

2. Tumour marker(CEA. CA 125 . CA 15.5)

3.BONE SCAN

4.ECHO

5.LABS

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

Meanwhile start NACT (Taxo.carbo every 3 weeks) +Zoladex

If proved non_metsadd Herceptin&perjeta ,otherwise re MDT.

SIGNATURE

MEDICAL ONCOLOGY

MEDICAL ONCOLOGY

د/ شيماء لاشين

RADIOLOGY

RADIOLOGY

د/ خالد

SURGERY

SURGERY

د/ محمد الهمادي

PATHOLOGY

PATHOLOGY

Psychiatric

Psychiatric

Nutrition

Nutrition

Name: NESMA MOHAMED SOLIMAN
Prof. Dr. NO DOCTOR
Pat. ID: 180434

Age : 38 Year(s)
Date : 25/11/2023
Study ID: 1450531

MRI OF THE PELVIS WITH CONTRAST

Technique:

- Sagittal T1 and T2-WIs.
- Axial T2-WIs.

Findings:

- Uterus: is surgically removed with clear stump.
- A well defined rounded, thin walled cyst is seen at the right adnexal region. It measures about 1.9x2.1cm diameters. The cyst contents is seen eliciting low T1 and high T2 signals with no soft tissue components and no abnormal enhancement.
- Normal MR appearance of the left *adnexa* with no focal lesions.
- Normal MR appearance of the *urinary bladder* with no focal lesions, or wall thickening.
- No abnormal *pelvic collection* or *pelvic masses* seen.
- No *pelvic lymphadenopathy* noted.
- No bony abnormalities detected with normal MR appearance of both *hip* and *sacroiliac joints*.

Opinion:

Right ovarian small follicular cyst for follow-up examination

Much Obligated
Prof. Hany Hafez Lotfy
Dr. Bassem Raouf, MD



وزارة الصحة والسكان
امانة المراكز الطبية المتخصصة
مستشفى اورام دار السلام

$$\frac{2 \times 7 \times 11}{22}$$

Name: نورة محمد سليمان
Date: 6/2/2025

Age:

EF	64%	54-74%	PWT	0.7	0.6-1.1cm
LVEDD	4.9	3.5-5.7cm	AO	3.4	2.0-3.7cm
LVESD	2.7	2.2-4.0cm	LA	3.5	1.9-4.0cm
FS	31%	25-45%	RV		0.7-2.3cm (supine)
SWT	0.9	0.6-1.1cm	AO.CUSP		1.5-2.6cm

mlt TR

D/Mohamed Ali



Patient Name: Nesma Mohamed Soliman Mohamed	Date : 21/12/2024
Exam : C.T CHEST WITH CONTRAST	Age : 39 Year(s)
Medical ID: 3868/2023	Pat. ID: 42995

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

*****Breast Cancer patient underwent left mastectomy.** As Compared to previous study dated (13/6/2024)***

Findings:

I Chest: -

- Clear operative bed showing no masses or cysts.
- Still noted:
 - Bilateral apical fine reticulations, and bilateral scattered atelectatic bands
 - Few bilateral scattered tiny pulmonary nodules some of them show internal calcifications, the largest is noted at apical segment of right upper lung lobe measuring 5 mm.
- Currently seen left upper lobe subpleural atelectatic bands and fine reticulations, Likely post radiation sequel.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- The liver is average in size with homogenous CT texture. No definite focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.



Patient Name: Nesma Mohamed Soliman Mohamed	Date : 21/12/2024
Exam : C.T CHEST WITH CONTRAST	Age : 39 Year(s)
Medical ID: 3868/2023	Pat. ID: 42995

- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- **Progressive course** regrading the size and extensions of the multiloculated right adnexal cystic lesion with internal enhancing septations seen currently measuring 9x9.7x14 in maximum AP, TS, and CC dimensions respectively compared to 5x5.6x9.5cm priorly.
- Normal appearance of the other pelvic organs.

The scanned skeleton:

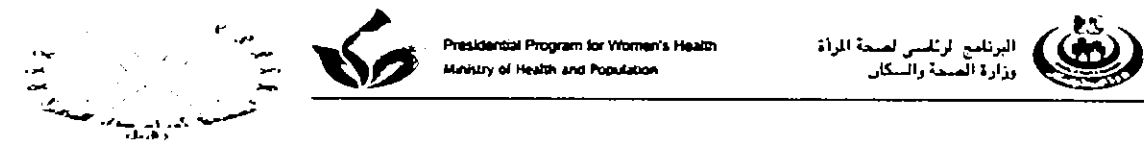
- Still noted right iliac wing and left pubic sclerotic foci mostly bone islands.
- L2 small sclerotic foci **for bone scan correlation.**
- No vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

- **Progressive course regrading right adnexal lesion.**
- **Stationary course of the rest of the study.**

Dr. Basma Mohamed

Dr. Walid Mohamed, M.D.



MDT form

A-Patient ID:
1- Name : نسمة محمد سليمان محمد

Sex:
Female

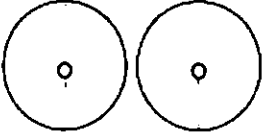
3- Medical number:
83167618

Old Med :
3868-2023

2-ID number: 28509070105743

Patient Age : 38 Y

BR. NACT
clinical CR



Family history	Family History	Phone	Phone	BMI	BMI	REASON OF MDT	REASON OF MDT POST NACT
			01062592976				
marital status	Marital Status	NRS Score	NRS Score	GROUP	GAC_P A	Date of MDT	Date of MDT 21/02/2024 ☒
Co morbidity	Co Morbidity free	Menopausal status	Menopausal Status PREMENOPAUSAL	Initial STAGE	Initial STAGE		

Clinical data

Breast Imaging :
LT breast locally advanced breast + ipsilateral axillary LN

Mets Workup :
CT CAP : FREE
large adenexial lesion mostly neoplastic
Bone scan : requested
transvaginal U/S :LARGE adenxal midline & to the rt side
complex cystic lesion

BX IDC	ER 0/8	PR 7/8	Her 2 Neu 3	SISH	Ki67 50
BX COMMENT:	ER COMMENT:	PR COMMENT:	Her 2 Neu COMMENT:	SISH COMMENT :	

T 4b	N 1	M 0
---------	--------	--------

Sonomammography

PATIENT HISTORY
finished 6 cycle TC + HP x4 10/2/2024

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

MRI : follicular ovarian cysts.
post NACT (clinical CR) for LT MRM.

SIGNATURE

MEDICAL ONCOLOGY	MED CAL ONCOLOGY د/شيماء يونس	RADIOLOGY	RADIOLOGY د/عبدالله بن يحيى
SURGERY	SURGERY د/محمد عبد المنعم	PATHOLOGY	PATHOLOGY
Psychiatric	Psychiatric	Nutrition	Nutrition



Patient Name	: الأستاذ / نسمة محمد سليمان محمد	Request No.	: 13003142
Patient No.	: 28509070105743	Request Date	: 17/02/2025
Path No.	: 686/25	Verification Date	: 09/03/2025
Sex	: Female	Financial Category	: نفقة الدولة - اورام دار السلام
Age	: 39 Y	Department	: Out Patient
Referring Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical data

Ovarian mass

Nature of specimen

Ovarian mass excision, omentectomy & appendectomy , small bowel resection

Gross specimen

Four specimens were received :

- 1) Right ovarian mass : an intact bilocular cyst measured 9.5x5x4.5 cm, outer surface was focally rough , on opening , it contains yellowish serous fluid , the lining was smooth showing cystification , wall thickness 0.3 cm, the outer surface showed disrupted fimbriated tubal segments measured 3.5 cm , also received in the same container multiple irregular tissue pieces measured collectively 5x2.5 cm , with grey brown & rim of glodend yellow cut section
- 2) Omental tissue piece , two omental tissue pieces measured 8x4 cm, and 4.5 x2 cm with no gross nodules or induration
- 3) Appendectomy specimen measured 4.5 long , ouetr surface was smooth & glistening , lumen was patent , maximum diameter 0.4 cm, average wall thickness 0.3 c,
- 4) Small intestinal segment measured 12 cm long with one end diameter 2 cm , the other end 1.5 cm , ouer surface was rough and congested , showed mutiple perforations , ranging in size from 1.5x1 cm , upto 2.5x2 cm, the smallest perforationn located 0.4 cm from narrower end, on opening , the mucosa showed thinned out area measured 2x2 cm, located 3 cm from wider end with hemorrhagic mucosal cystification

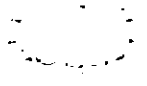
Microscopic examination

-histopathological examination of sections prepared from the ovarian cystectomy received reveals a serous cyst lined by low cuboidal cells. The wall shows sma l papillae with broad fibrous cores covered by same type of epithelium. There is no evidence of malignancy in

Verified by :

Dr.Heba Hasan





Patient Name : الأستاذة / نسمة محمد سليمان محمد	Request No. : 13003142
Patient No. : 28509070105743	Request Date : 17/02/2025
Path No. : 686/25	Verification Date : 09/03/2025
Sex : Female	Financial Category : نفقة الدولة - اورام دار السلام
Age : 39 Y	Department : Out Patient
Referring Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

sections examined.

- Para tubal cyst
- Ovarian tissue with hemorrhagic corpus leuteum cyst
- Congested omentum
- histopathological examination of sections prepared from the appendectomy specimen received shows partially ulcerated mucosa, congested oedematous submucosa and hyperplastic submucosal lymphoid follicles. There is no evidence of specific inflammation or neoplasia in sections examined.
- Hemorrhagic intestinal mucosa with angiodysplasia
- No malignancy

Diagnosis

- Right ovarian mass, excision, & omentectomy & appendectomy :-Ovarian serous cystadenoma
- Paratubal cyst
- Ovarian tissue with hemorrhagic corpus leutum
- Congested omentum
- Follicular appendicitis
- Hemorrhagic intestinal mucosa with angio dysplasia
- No malignancy

Signature
Dr. Rana Fathy
Dr.Heba Hassan

Verified by :
Dr.Heba Hasan



٢٠١٩/١٠ ٢٨٥٠٩٠٧٠١٠٥٧٤٣
حاصله على دبلوم تجارة
أنشى مسلمة متزوجة
بخت الضوى عبد الحميد محمد
البطاقة سارية حتى ٢٠٢٦/١٠/٠٧



جمهورية مصر العربية
بطاقة تحقيق الشخصية
نسمة
محمد سليمان محمد ابراهيم
١٠ ش وابور الطحين - عزبة الوابور
المطرية - القاهرة
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